Cleveland Clinic

INTERNATIONAL & EMERGING MARKETS | ADVISORY OFFERING PROFILE

Integrated Financial Planning, Forecasting & Modeling

An advisory offering that helps finance teams build driver-based plans, budgets and forecasts that connect volumes, staffing, revenue and capital.

CATEGORY
Finance Advisory: Manage Stage (planning, budgeting and forecasting with confidence)

TYPICAL DURATION
2 to 12 weeks per module; 12 to 20 weeks for the full toolkit

DELIVERY MODEL
Hybrid, Workshop-based

01 CLIENT NEED

Volume assumptions drive nearly every downstream plan, from staffing and physician compensation to revenue, capital and cash. Yet many organizations plan with one-off spreadsheets that cannot be reused, audited or scenario-tested, budget processes that are lengthy and disconnected from operations, and assumptions that are undocumented or inconsistent. Boards and executives need an integrated, driver-based view of the plan that shows whether it is affordable, how it responds to change and how it will be managed through the year.

This offering sits in the Manage stage of the Cleveland Clinic Healthcare Financial Advisory Lifecycle, which follows an organization's financial decisions through five stages: Plan, Measure, Optimize, Manage and Transform. The Manage stage covers planning, budgeting and forecasting with confidence, and answers the client question "How do we improve planning and decision-making?" It is typically requested by multi-site organizations and providers facing budgeting or forecasting challenges.

02 CLEVELAND CLINIC VALUE PROPOSITION

This offering is delivered by Cleveland Clinic's Finance Advisory team. Cleveland Clinic is an operating health system with 23 hospitals, 300 outpatient locations and about 83,000 caregivers, and has grown from Northeast Ohio into Florida, Las Vegas, Toronto, London and Abu Dhabi (figures as of December 31, 2025).

Clients get models built around the drivers that matter in a working health system, drawing on how Cleveland Clinic plans, budgets and forecasts across a large, complex academic health system. Volumes,

reimbursement, staffing and capital are linked the way they interact in real hospital operations.

Models are built with the client's finance team and handed over with training, documentation and an assumptions workbook, so the team can own, update and extend them after the engagement.

03 KEY DELIVERABLES

Each model or analysis below can be delivered as a standalone module. Together they form an integrated planning toolkit, built volume and revenue first.

- **Volume and demand model:** Excel-based model by service line, with service phasing and a ramp-up schedule
- **Revenue and reimbursement model:** Excel-based model built on chargemaster pricing, payer mix and contract terms
- **Staffing and workforce cost model:** Excel-based model with volume-driven full-time equivalent (FTE) calculations, organizational structure and physician compensation scenarios
- **Integrated three-statement model:** Excel-based profit and loss, cash flow and balance sheet, with scenario toggles and an output dashboard
- **Assumptions workbook:** inflation, service mix, pricing and recruitment assumptions, each with its source, owner and update cadence
- **Scenario and sensitivity package:** best, base and downside cases, with tornado charts showing which assumptions matter most and break-even analysis
- **Operating budget package:** redesigned budget process with templates, roles and an annual calendar
- **Multi-year forecast:** 3 to 10 year projection with a predictive forecasting method and accuracy tracking
- **Capital plan:** prioritization framework, return on investment (ROI) analysis for each request, a standard request process and capitalization policy recommendations

04 DELIVERY MODEL

Hybrid / Workshop-based

Delivered by Cleveland Clinic's Finance Advisory team, working alongside the client's finance team. Design workshops set the model structure, drivers and assumptions with finance leadership. Models and processes are then built iteratively, reviewed in weekly virtual sessions and validated against historical results. The engagement closes with training and documentation so the client team can own and update the work.

05 TYPICAL DURATION

2 to 12 weeks per module; 12 to 20 weeks for the full toolkit

- **Design and assumption setting (2 to 3 weeks):** Workshops to agree model structure, drivers and assumptions with finance leadership
- **Build and review (8 to 14 weeks):** Models and processes built iteratively, volume and revenue drivers first, with weekly review sessions
- **Validation, training and handover (2 to 3 weeks):** Results tested against history, then training and documentation for the client's team

Phase timing above is for the full toolkit (12 to 20 weeks). A single module follows the same phases: focused analyses such as a scenario package, operating budget or capital plan take 2 to 6 weeks, and individual driver or statement models take 6 to 12 weeks. The range depends mainly on the number of models, sites and service lines in scope, the projection horizon and the quality of historical data.

06 RESOURCE REQUIREMENTS

CLEVELAND CLINIC TEAM

- Director / Executive Sponsor: 5 to 10% (larger engagements)
- Assistant Director / Engagement Lead: 10 to 25%
- Manager / Workstream Lead: 20 to 40% (mid-sized and larger engagements)
- Senior Analyst: 30 to 60%
- Analyst: 20 to 50%
- Admin / Project Support: up to 10%

CLIENT TEAM

- Executive sponsor with decision authority, for scoping, key reviews and the final readout
- Finance lead as day-to-day point of contact, attending the weekly working sessions
- Finance analysts who will own and maintain the models after handover
- Operational leaders to validate volume, staffing and capital assumptions
- IT or data team support for system extracts

Percentages are the share of each team member's working time during the engagement, so they do not add up to 100%. Roles marked for larger engagements join only when the scope calls for them.

ENGAGEMENT ASSUMPTIONS

- The client provides timely access to the financial, operational, market, volume, stakeholder, reporting and staffing information in the data request

- Scope and timing may change if data is late or incomplete
- Modules and the projection horizon are confirmed at scoping
- Stakeholders are available for interviews and working sessions, and draft deliverables are reviewed within an agreed turnaround
- Recommendations are advisory; decisions and implementation remain with the client's leadership

07 EXPECTED OUTCOMES

- Finance and operations work from one shared, driver-based model, so staffing, revenue and capital decisions tie to the same volume forecast
- Leadership can test scenarios quickly and see downside exposure before committing to a plan
- The board and lenders receive an integrated view of profitability, cash and financial position
- The budget cycle is shorter, and operational leaders own their targets
- Capital requests are prioritized against funding capacity through one consistent process
- The client's finance team maintains and extends the models after handover, supported by training, documentation and an assumptions workbook with named owners